

Tesofensine

Tesofensine — Triple Monoamine Reuptake Inhibitor

WEIGHT LOSS

APPETITE
SUPPRESSION

DOPAMINE

NOREPINEPHRINE

SEROTONIN

WHAT IS IT?

Tesofensine is a novel triple reuptake inhibitor of dopamine, norepinephrine, and serotonin (similar mechanism to phentermine but more selective). Originally developed for Parkinson's and Alzheimer's disease, Phase 2 weight loss trials showed 10.6% body weight reduction at 0.5 mg/day over 24 weeks — outperforming most approved anti-obesity drugs of its era. It reduces appetite through multiple central pathways simultaneously.

DOSING AT A GLANCE

Starting dose	Standard dose	Max dose (caution)	Timing
0.25 mg/day	0.5 mg/day	1.0 mg/day	Morning

KEY BENEFITS

- 10.6% body weight reduction at 0.5 mg/day (Phase 2 TIPO-1 trial, 24 weeks)
- Significant reduction in appetite and food cravings
- Increases basal metabolic rate (norepinephrine effect)
- Improved energy and motivation (dopamine + norepinephrine)
- Mood enhancement (serotonin component)
- Superior to most single-mechanism appetite suppressants
- Synergistic with GLP-1 agonists for combined central + peripheral appetite control
- Shown to reduce caloric intake by 15–20% in studies

HOW IT WORKS

Tesofensine blocks the reuptake transporters for dopamine (DAT), norepinephrine (NET), and serotonin (SERT) — increasing synaptic levels of all three monoamines. Norepinephrine increases energy expenditure and reduces appetite via beta-adrenergic receptors. Dopamine reduces reward-driven eating. Serotonin signals satiety centrally. The triple mechanism prevents receptor adaptation that limits single-mechanism drugs.

PROTOCOLS

- **Start low:** 0.25 mg orally daily x4 weeks, then titrate to 0.5 mg.

- **Standard protocol:** 0.5 mg/day morning, assess at 8 weeks for response.
- **Cycling:** 12–16 weeks on, 4–8 weeks off to prevent monoamine depletion.
- **Stack:** Tesofensine + GLP-1 agonist (semaglutide/tirzepatide) for additive weight loss.
- **Do not stack with:** MAOIs, SSRIs, SNRIs (serotonin syndrome risk).

SIDE EFFECTS

- Dry mouth (very common — most frequently reported side effect)
- Nausea (common early, diminishes)
- Increased heart rate (norepinephrine effect — monitor cardiovascular status)
- Insomnia if taken late in day
- Constipation
- Elevated blood pressure (monitor BP weekly during titration)
- Psychological dependence risk with long-term use (dopamine mechanism)

STORAGE

Oral capsules/powder — room temperature, away from moisture and light. Stable 2+ years sealed.

■ **Cardiovascular risk — contraindicated in uncontrolled hypertension, arrhythmia, or structural heart disease. Not for use with other serotonergic drugs.**

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